CLARITY

REAL INSIGHTS IN SECONDS

THE WEEK AT A GLANCE

Cardiology

October 30, 2026 · 3 selected publications

CURATED
Only the essentials

01

MULTICENTER RCT · 14 HOSPITALS · N = 499 RANDOMIZED

Left Bundle-Branch Area Pacing vs Right Ventricular Pacing for Atrioventricular Block and Preserved Left Ventricular Function: The LEAP-Block Randomized Clinical Trial

JAMA · Oct 28, 2026

WHY IT MATTERS

The LEAP-Block trial randomized patients with atrioventricular block and preserved LVEF (≥50%) to left bundle-branch area pacing (LBBAP) or right ventricular pacing (RVP) and compared 24-month clinical outcomes. According to the abstract, the optimal pacing strategy to reduce heart failure and all-cause mortality in patients requiring high-burden ventricular pacing remains uncertain, and robust comparative evidence is needed.

KEY FINDING

Of 499 randomized patients, 482 entered the primary analysis (LBBAP n = 242; RVP n = 240; mean age 67 years; 46% female). The primary end point occurred in 2.4% with LBBAP vs 12.5% with RVP (absolute risk difference -10.1% [95% CI, -16.8% to -3.4%]; P = .003). HF events were 3.4% vs 11.3% (-7.9% [95% CI, -13.0% to -2.8%]). The 24-month Kaplan-Meier estimated cumulative incidence of the primary end point was 5.0% vs 13.2% (hazard ratio, 0.37 [95% CI, 0.19 to 0.73]; P = .004). Acute procedure success was 97.9% vs 99.6% (P = .22). No significant differences were observed in all-cause death, biventricular pacing upgrade, device-related complications, or pacing-induced cardiomyopathy.

2.4% vs 12.5%	HR 0.37	3.4% vs 11.3%
PRIMARY END POINT, 24 MO	KAPLAN-MEIER PRIMARY END POINT	HF EVENTS
LBBAP vs RVP; P = .003	95% CI 0.19 to 0.73; 5.0% vs 13.2%	LBBAP vs RVP; 95% CI -13.0% to -2.8% for difference

PMID 42949742 · View on PubMed

02

IPD META-ANALYSIS OF 5 RCTS · 110,933 PATIENT PRESENTATIONS

Performance of 0/1 h versus 0/3 h pathways in patients with suspected myocardial infarction: a systematic review and individual participant data meta-analysis of randomised controlled trials

The Lancet · Oct 29, 2026

WHY IT MATTERS

This systematic review and individual participant data meta-analysis pooled all randomised trials comparing the 0/1 h pathway with the 0/3 h pathway (standard care) in patients with suspected myocardial infarction, using harmonised outcome definitions. According to the background, the 0/1 h pathway is recommended in current guidelines, but individual trials used different safety outcomes and efficacy outcomes were inconclusive or not assessed.

KEY FINDING

Five randomised trials, all providing individual participant data, were included, comprising 110,933 patient presentations (55,489 with the 0/1 h pathway and 55,444 with the 0/3 h pathway). All-cause death or new type 1 myocardial infarction within 30 days occurred in 446 (0·8%) versus 562 (1·0%) patient presentations (pooled OR 0·83 [95% CI 0·45-1·53]; I2=62·9%; 2=0·143). Emergency department length of stay was 314 min (IQR 211-481) versus 308 min (209-453) (pooled geometric mean ratio 1·03 [95% CI 0·85-1·25]; I2=99·2%). Direct discharge occurred in 67·0% versus 65·8% (pooled OR 1·31 [95% CI 0·79-2·16]; I2=97·8%).

0·8% vs. 1·0%	314 vs. 308 min	67·0% vs. 65·8%
30-DAY DEATH/TYPE 1 MI	ED LENGTH OF STAY	DIRECT ED DISCHARGE
pooled OR 0·83, 95% CI 0·45-1·53	geometric mean ratio 1·03, 95% CI 0·85-1·25	pooled OR 1·31, 95% CI 0·79-2·16

PMID 42956796 · View on PubMed

03

POST HOC RCT ANALYSIS · SMART-CHOICE 3 · N = 5,506

Clopidogrel Versus Aspirin Monotherapy According to Atherothrombotic Risk Profile

Circulation: Cardiovascular Interventions · Oct 29, 2026

WHY IT MATTERS

This post hoc analysis of the SMART-CHOICE 3 randomized trial compared clopidogrel with aspirin monotherapy in patients who completed dual antiplatelet therapy after PCI, stratified by atherothrombotic risk (TIMI Risk Score for Secondary Prevention and complex PCI). The background states that it was unclear whether treatment effects differ by overall atherothrombotic risk integrating clinical and angiographic factors.

KEY FINDING

Of 5,506 patients, 4,486 (81.5%) were classified as higher risk and 1,020 (18.5%) as lower risk. Over a median 2.3 years, MACCE occurred in 6.1% of higher-risk versus 3.1% of lower-risk patients (HR 1.83 [95% CI 1.17-2.78]; P=0.007). In higher-risk patients, MACCE was 4.6% with clopidogrel versus 7.4% with aspirin (HR 0.67 [95% CI 0.50-0.89]; P=0.006). In lower-risk patients, it was 3.0% versus 3.2% (HR 1.03; P=0.945; interaction P=0.242). Bleeding (BARC type 2, 3, or 5) was similar between groups.

4.6% vs. 7.4%	6.1% vs. 3.1%	3.0% vs. 3.2%
MACCE, HIGHER-RISK PATIENTS	MACCE BY RISK GROUP	MACCE, LOWER-RISK PATIENTS
Clopidogrel vs. aspirin; HR 0.67 (95% CI 0.50-0.89)	Higher vs. lower risk; HR 1.83 (95% CI 1.17-2.78)	Clopidogrel vs. aspirin; HR 1.03; P=0.945

PMID 42953942 · View on PubMed

SOURCE STANDARDS

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